

Operative Report

GI Associates of Boston | Dr. Elena Hartley, MD

Patient: Jane Doe | DOB: 1988-03-14 | MRN: 00142837

Date of procedure: 2022-03-14 Surgeon: Dr. R. Hoffman, MD, FACS

Anesthesia: General Duration: 45 min EBL: 20mL

PREOPERATIVE DIAGNOSIS

Symptomatic hemorrhoids and anal fissure, likely exacerbated by chronic diarrhea from UC.

PROCEDURE PERFORMED

Hemorrhoidectomy (Ferguson technique) x2 columns, lateral internal sphincterotomy.

OPERATIVE DETAILS

Patient placed in prone jackknife position under general anesthesia. Perineum prepped and draped in sterile fashion. Digital rectal exam performed -no masses. Anoscopy confirmed grade III internal hemorrhoids at 3 and 7 o'clock. Chronic posterior fissure noted.

Hemorrhoidectomy: Ferguson (closed) technique performed at 3 and 7 o'clock positions. Hemorrhoidal tissue excised to dentate line. Wounds closed with 3-0 chromic suture.

Sphincterotomy: Lateral internal sphincterotomy performed at 9 o'clock. Fissure margins freshened. Hemostasis achieved.

Patient tolerated procedure well. Extubated in OR, transferred to PACU in stable condition.

POSTOPERATIVE PLAN

Sitz baths TID, stool softeners, high-fiber diet when UC allows.

Follow-up in 2 weeks. Pathology sent on hemorrhoidal tissue.